

CORY SHEPHERD

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Supporting statement – two applications for sick leave without pay

Recoding of two periods presently recorded as leave without pay, at the request of Metro South Payroll · 11 September 2026

I apply for **sick leave without pay** for the two periods identified by Metro South Payroll on 7 and 8 September 2026. The dates below are Payroll's own and are not varied.

Application	Period	Days (inclusive)
1	21 June 2024 – 20 September 2024	92
2	13 December 2024 – 23 February 2025	73

Both periods are presently recorded as leave without pay. No other period is affected.

Payroll has already identified the correction. On 8 September 2026 Ms Grace Strachan, Senior Payroll Officer, wrote that these periods "should have been booked as Sick No Pay (SNP) rather than Leave Without Pay (LWOP)", that "new leave forms will be required for both periods", and that once the forms are received and the leave updated, "we can recognise the first three months of each period towards your LSL date." These two applications supply the leave forms Payroll requires. I am not asking anyone to accept a new position; the position is Payroll's own.

Both absences were medical. Four work capacity certificates are attached – Dr Peter Hawes 1 July 2024, Dr Ki Pang 7 August 2024, and Dr Peter Hawes 11 August and 8 September 2024. They record the stated date of injury as 18 June 2024, first presentation on 1 July 2024, no pre-existing factor or condition, and **no functional capacity for any type of work over the period 1 July to 6 October 2024**. The second period was a continuation of the same condition, under the care of a treating specialist from 24 October 2024, and it ends on 23 February 2025 – the day before my return to work at 7:00 am on 24 February 2025. If anything further is required, please tell me what it is and I will provide it.

Effect. Payroll records commencement on 25 March 2019 and 189 days presently excluded, moving the long service leave date to 29 September 2026. Each of these two periods falls entirely within its own first three months, so on Payroll's stated approach 165 of those 189 days return. The calculation is of course Payroll's to make.

Reservation. I have received no wages for work since 13 July 2026. Any leave applied for, taken or paid since that date is applied for and taken under protest and without prejudice to my claim for wages, and I reserve the right to seek reinstatement of the leave taken. These two applications relate to 2024 and 2025 and are made solely to give effect to the recoding Payroll has identified.

Signed



Cory Lea Shepherd · Administration Officer, Switchboard Services, Logan Hospital · Employee no. 388372

Date: 11 September 2026

Attached: work capacity certificates of Dr Peter Hawes dated 1 July 2024, Dr Ki Pang dated 7 August 2024, and Dr Peter Hawes dated 11 August and 8 September 2024.

Work capacity certificate – workers' compensation

Form 132M – Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work.

Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name Cory Shepherd Date of Birth 11/01/1991
Mobile number 0422438627 Claim number S23LW142013 ☒ New claim ☐ Claim is report only
Occupation (if known) Admin worker Patient's employer Qld Health

Part B - Injury details

Date of examination 01/07/2024 Patient's stated date of injury 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on 01/07/2024

The patient is/was suffering from (List all work-related diagnoses. If symptoms only, tick "Provisional diagnosis") ☐ Provisional diagnosis anxiety, stress

Patient's stated mechanism of injury [redacted] Is this consistent with your clinical findings? ☒ Yes ☐ Unclear

Describe mechanism in detail [redacted]
Pre-existing factors or condition aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 04/08/2024 to be reviewed again on 04/08/2024 ☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline Details (specify)
Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious,

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval
I have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Peter Hawes Email admin@ourmedicalashmore.com.au
Practice/hospital Our Medical Ashmore Phone 0756475000 Date 01/07/2024
Postal Address Signature
566 Olsen Avenue, Ashmore
QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

All enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2016, pursuant to section

586 of the Act. © State of Queensland (Queensland Treasury) 2016



Work capacity certificate – workers' compensation

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION Work is an important part of recovery. In most cases, a practitioner's guidance increases the likelihood of positive return to work outcomes. Consider the health benefits of work when certifying the patient's capacity.

Early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner receiving continued support is three times more likely to regain their capacity to work.

Part A - Patient details

Name Cory Shepherd
Mobile number 0422438627
Occupation (if known) Admin worker

Claim number S23LW142013

Date of Birth 11/01/1991

☒ New claim

Patient's employer Qld Health

☐ Claim is report only

Part B - Injury details

Date of examination 01/07/2024

Patient's stated date of injury 18/06/2024

Patient was first seen at this practice/hospital for this injury/disease on

01/07/2024

The patient is/was suffering from (List all work-related diagnoses, if symptoms only, tick "Provisional diagnosis")

☐ Provisional diagnosis

anxiety, stress

Patient's stated mechanism of injury

Is this consistent with your clinical findings?
☒ Yes ☐ Unclear

Describe mechanism in detail
Pre-existing factors or condition

aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 09/08/2024 to be reviewed again on 09/08/2024

☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function

☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Specialist/GP

Name/discipline

Details (specify)

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F.

☐ If suitable duties available, can return to some form of work from

☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	No Yes Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work life assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval
I have discussed injury requirements and return to work with the patient and ☐ Employer

Part G - Medical/dental/nurse practitioner details and statement (for use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Ki Pang

Practice/hospital Our Medical Ashmore

Postal Address

588 Olsen Avenue, Ashmore, QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

All enquiries (medical/dental/nurse practitioner, patient, employer) 131 362 124

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation claimant's application for compensation. The information contained in this form may be disclosed to the insurer, another insurer, medical or allied health professional, or other medical, allied health, or rehabilitation provider. The claimant may be contacted by the insurer or the Workers' Compensation Regulator on 31 May 2016.

666 of the Act © State of Queensland (Queensland Treasury) 2016

Work capacity certificate – workers' compensation

Form 132M Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work. Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name: Cory Shepherd Date of Birth: 11/01/1991
Mobile number: 0422438627 Claim number: S23LW142013 ☐ New claim
Occupation (if known): Admin worker Patient's employer: Qld Health ☐ Claim is report only

Part B - Injury details

Date of examination: 11/08/2024 Patient's stated date of injury: 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on: 01/07/2024

The patient is/was suffering from (List all work-related diagnoses): anxiety, stress

If symptoms only, tick "Provisional diagnosis" ☐ Provisional diagnosis

Patient's stated mechanism of injury: [redacted] Is this consistent with your clinical findings?
☒ Yes ☐ Unclear

Describe mechanism in detail

Pre-existing factors or condition

aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 11/08/2024 to 08/09/2024 to be reviewed again on 08/09/2024 ☐ No further review

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline: Details (specify):

Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until 8/9/24

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious,

Estimated time to return to some form of work duties: unknown

Estimated time to return to full duties: unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

I require a suitable duties program to be provided to me for approval

have discussed injury requirements and return to work with the patient and ☐

Employer

☐ Insurer

☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name: Peter Hawes

Practice/hospital: Our Medical Ashmore

Postal Address:

6 Olsen Avenue, Ashmore

QD 4214

Email: admin@ourmedicalashmore.com.au

Phone: 0756475000 Date: 11/08/2024

Signature: [Signature]

Further information: www.worksafe.qld.gov.au/medicalsupport

enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act) the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2018, pursuant to section 585 of the Act. © State of Queensland (Queensland Treasury) 2016



Work capacity certificate – workers' compensation

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and well-being. Practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity. Consider the health benefits of work when setting the patient's capacity.

Part A - Patient details

Name Cory Shepherd

Mobile number 0422438627

Occupation (if known) Admin worker

Claim number S23LW142013

Date of Birth 11/01/1991

☐ New claim

Patient's employer Qld Health

☐ Claim is report

Part B - Injury details

Date of examination 11/08/2024

Patient's stated date of injury 18/06/2024

Patient was first seen at this practice/hospital for this injury/disease on 01/07/2024

The patient is/was suffering from (List all work-related diagnoses. If symptoms only, tick "Provisional diagnosis") Provisional diagnosis

anxiety, stress

Patient's stated mechanism of injury

Is this consistent with your clinical findings? ☐ Unclear

Describe mechanism in detail

Pre-existing factors or condition

aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 08/09/2024 to 06/10/2024 to be reviewed again on 06/10/2024

Treatment needs time away from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Specialist/GP

Name/discipline

Psychiatrist

Details (Specify)

Part D - Capacity for work

☐ Time certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F.

☐ If suitable duties available, can return to some form of work from

☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work. If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)

stressed and anxious.

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)

Is functional ability affected by injury/condition?

No

Yes Note any restrictions (if relevant)

What patient can do (if "yes" is selected)

Lower limb

No

Upper limb

No

Hand function

No

Spinal function

No

Cognition/psychosocial functioning

No

Driving a car

No

Operating machinery/heavy vehicle

No

Manual tasks

No

Other

No

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.) What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

I require a suitable duties program to be provided to me for approval

We discussed injury requirements and return to work with the patient and

☐ Employer

☐ Insurer

☐ Rehabilitation

G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate

Peter Hawes

Address Our Medical Ashmore

Address Avenue, Ashmore

Email admin@ourmedicalashmore.com.au

Phone 0756475000 Date 08/09/2024

Signature

Information www.worksafe.qld.gov.au/medicalsupport

(medical/dental/nurse practitioner, patient, employer) 1300 362 128

Workers' Compensation and Rehabilitation Act 2003 (the Act) the workers' compensation insurer is authorised to collect the information on this form to process the claim for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or other persons in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 24 April 2023.

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